

PATIENT SCREENING REPORT

Retinal Edge Triage — Explainable AI-Assisted Diabetic Retinopathy Screening for Rural Healthcare
Screening Date & Time: 05-Sep-2026 13:35 UTC | Reference ID: TEST-001

Patient ID: PID-1	Full Name: Test Patient	Age / Gender: N/A / N/A
Screening ID: TEST-001	Location: Rural PHC	Diabetes Type / Duration: Type 2 (N/A)

1. Retinal Image Acquisition & Quality Gate

Eye Evaluated: Right Eye	Image Source: Fundus Camera	Quality Score: 92.0%	Quality Status: GOOD
Focus / Blur: 88.0%	Illumination: 92.0%	Vessel Visibility: 91.0%	Field of View: 94.0%

2. Diabetic Retinopathy Screening & Detection Result

DR Detection Result:	Diabetic Retinopathy: DETECTED	DR Grade & Severity:	Grade: 3 — Severe NPDR
Model Confidence:	94.0%	DME Risk Status:	Possible DME detected
Result Explanation:	Severe non-proliferative diabetic retinopathy with multi-quadrant hemorrhages (8) and vascular beading.	Recommended Action:	Severe diabetic retinopathy requires urgent ophthalmic evaluation within 1 month.

3. Human-in-the-Loop Ophthalmologist Adjudication

Doctor Decision:	PENDING_REVIEW	Reviewing Doctor:	Dr. Rajesh Varma, MS
Final Grade Assigned:	Grade 3	Override Justification:	None / Matched AI assessment
Clinical Notes:	Clinical findings consistent with diabetic retinopathy changes.	HCW Instructions:	Proceed to patient referral counselling.

4. Risk Triage & Clinical Disposition

Risk Category:	HIGH RISK	Referral Priority:	URGENT
Referral Reference ID:	REF-2026-PENDING	Follow-up Schedule:	3 Months
Clinical Justification:	Severe diabetic retinopathy requires ophthalmic evaluation.	Referral Status:	PENDING

5. Recommended Eye-Care Facility (Referral Destination)

Hospital / Clinic Name:	ABC Eye Hospital	Distance:	2.4 km away
Hospital Address:	Sayyaji Rao Road, Mysuru	Contact Number:	98765 43210

Reason for Referral:	Severe diabetic retinopathy requires ophthalmic evaluation.	Navigation / Directions:	View Map & Directions
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STATUTORY CLINICAL NOTICE: This report is generated by an artificial intelligence screening triage prototype (Retinal Edge Triage). AI predictions DO NOT constitute a definitive medical diagnosis. Final clinical interpretation and management must always be performed by a qualified ophthalmologist. Compliant with rural screening clinical governance principles.